

August 20, 2026

UnitedHealthcare
Provider Appeals
P.O. Box 30432
Salt Lake City, UT 84130

RE: Formal Appeal of Claim Denial — Malik Ward (MRN MUSC6249242), Claim MUSC-C36D-2

Patient	Malik Ward (DOB 1957-04-24)
MRN	MUSC6249242
Member ID / Group	UHC725804182 / GRP-83998
Claim number	MUSC-C36D-2
Date of service	2026-03-04
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$403.49
Denial code	CARC 197 / RARC N54 — Precertification/authorization/notification/pre-treatment absent.
Appeal deadline	2027-04-28

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this formal first-level appeal on behalf of Malik Ward (Member ID: UHC725804182, Group: GRP-83998, MRN: MUSC6249242, DOB: 1957-04-24) regarding the denial of Claim MUSC-C36D-2 for services rendered on March 4, 2026, at MUSC Health Rutledge Tower Ambulatory Care. UnitedHealthcare denied the claim in full on April 28, 2026, citing CARC 197 ("Precertification/authorization/notification/pre-treatment absent") and RARC N54 ("Claim information is inconsistent with pre-certified/authorized services"), with the payer remark stating: "No prior authorization on file for the date of service billed." MUSC Health respectfully disputes this denial on the grounds that the service rendered — an established outpatient office visit for an acute upper respiratory condition — does not require prior authorization under applicable plan terms and standard commercial payer guidelines governing urgent, low-acuity ambulatory encounters.

CLINICAL BACKGROUND

Mr. Ward is a 69-year-old male with an active medical history that includes atrial fibrillation, managed long-term with Verapamil Hydrochloride, Digoxin, and Warfarin Sodium, as well as obesity (BMI 30.55 kg/m², ICD-10 E66.9). He presented on March 4, 2026, to Dr. Daniel R. Okafor, MD (NPI 1558493021) at MUSC Health Rutledge Tower Ambulatory Care, an on-campus outpatient hospital facility (Place of Service 22). The encounter was an established patient visit billed as CPT 99214, reflecting moderate medical decision-making, with a primary diagnosis of acute upper respiratory infection, unspecified (ICD-10 J06.9) and a secondary diagnosis of obesity (ICD-10 E66.9). The total billed amount was \$403.49. Given Mr. Ward's underlying cardiac conditions and his anticoagulation regimen, clinical evaluation of an acute respiratory illness by his treating physician represents a medically appropriate and necessary service.

BASIS FOR APPEAL

Prior authorization requirements under commercial HMO plans are generally reserved for elective procedures, specialist referrals requiring gatekeeper approval, or high-cost diagnostic and surgical services. An established patient office visit for evaluation and management of an acute upper respiratory infection is a routine, time-sensitive ambulatory service for which imposing a prior authorization requirement would be both clinically impractical and inconsistent with broadly accepted managed care standards. Acute illnesses, by their nature, do not afford patients or providers the opportunity to seek prospective authorization before care is delivered. Requiring prior authorization for such encounters would create barriers to medically necessary care and is not consistent with the intent of utilization management programs.

Furthermore, CPT 99214 is a standard evaluation and management code for established outpatient patients presenting with moderate complexity. The service was rendered by a credentialed provider at a contracted MUSC Health facility during an active coverage period (January 1, 2024, through December 31, 2026). No clinical or coding irregularity has been identified that would independently support denial. The payer's own remittance reflects an allowed amount of \$256.25, which indicates the service itself is a covered benefit — the sole stated basis for non-payment is the absence of prior authorization. MUSC Health respectfully asserts that prior authorization was not required for this category of service and that the denial therefore lacks a valid contractual or clinical basis.

REQUESTED ACTION

MUSC Health requests that UnitedHealthcare conduct a full reconsideration of Claim MUSC-C36D-2 and issue payment at the previously established allowed amount of \$256.25, consistent with the applicable contractual rate for CPT 99214 rendered at Place of Service 22. Should the plan maintain that prior authorization was required for this encounter, MUSC Health further requests that UnitedHealthcare provide, in writing, the specific plan provision or utilization management policy that mandates precertification for established patient evaluation and management visits for acute illness, so that any administrative process gap may be addressed prospectively. This appeal is submitted within the plan's stated 365-day filing window and via the required electronic reconsideration channel.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record